

EXECUTIVE SUMMARY

300 staffed beds | 876,000 documentation events | 80% utilization

Nursing documentation is different from physician documentation—and so is the ROI model.

In outpatient and ED settings, the value story is relatively straightforward: time saved converts to visits, wRVUs, and revenue. In nursing, the story is more complex. Time saved doesn't generate revenue directly—it changes how nurses spend their shift, which cascades into retention, agency costs, patient safety, and experience scores.

This assessment models that cascade. We start with what's most measurable (overtime, retention, agency spend) and work toward outcomes that are real but harder to attribute directly (HAPI prevention, falls, HCAHPS). We're transparent about where we're confident and where we're being conservative.

The goal isn't to hand you a number. It's to give you a framework for thinking about value—one you can stress-test with your CNO, your finance team, and your frontline nursing leaders.

\$27K

NET ANNUAL VALUE

1.1x

ROI MULTIPLE

\$91

VALUE PER BED

\$250K

POTENTIAL VALUE

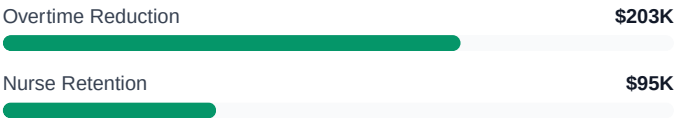
WHERE NURSING VALUE COMES FROM

**Labor & Operational** — Direct cost savings you can track in payroll and staffing data. Overtime goes down. Agency utilization drops. Turnover slows. These are measurable month-over-month.

**Quality & Safety** — Cost avoidance through better outcomes. Fewer HAPIs. Fewer falls. These are real, but the causal link to documentation is indirect—we show them as "potential value" to be intellectually honest.

**Qualitative** — Benefits that matter (HCAHPS, care coordination) but aren't included in the ROI total. They're part of the full picture, not the financial model.

LABOR COST BENEFITS (\$297K)



QUALITY & SAFETY BENEFITS (\$250K)



About Potential Value

Quality & Safety benefits (HAPI Prevention, Falls Prevention) are shown separately because the causal link is indirect. More bedside time ' better assessments ' fewer adverse events. The mechanism is clear, but attribution is complex. We include these as "potential value" rather than in the main ROI calculation.

QUALITATIVE BENEFITS (NOT QUANTIFIED)

- Survey & Compliance Readiness
- Care Coordination
- Patient Experience (HCAHPS)

These outcomes matter—they're just harder to quantify with confidence. See "The Full Picture" page for details.

YOUR INVESTMENT

STAFFED BEDS  
**300**

MONTHLY COST  
**\$225/staffed bed**

ANNUAL INVESTMENT  
**\$270K**

NET VALUE CREATED  
**\$27K**

## LABOR COST BENEFIT

## Overtime Reduction

**\$203K annually**

## WHY THIS MATTERS

Every nursing unit has a rhythm: shift start, rounds, assessments, medications, documentation, handoff, shift end. In theory, documentation fits within the shift. In practice, it doesn't.

Nurses routinely stay 30-60 minutes past their scheduled shift to finish charting. Multiply that across your FTEs, 5 days a week, 50 weeks a year—and you're looking at tens of thousands of overtime hours. At 1.5× pay, that's real budget impact.

But here's the thing: not all overtime is documentation. Some is patient acuity. Some is understaffing. Some is just the nature of nursing. We estimate ~33% of overtime is specifically end-of-shift charting catch-up—the kind that real-time ambient documentation can address.

This is the most measurable driver in nursing. You can track it in payroll data, week over week, unit by unit. If overtime drops after Abridge deployment, you'll see it.

## HOW WE CALCULATE THIS

## Step 1: Daily OT Per Nurse

*How much overtime does documentation drive?*

15 min/shift = 15 min overtime

*Documentation catch-up typically adds 15-30 minutes per shift.*

## Step 2: Annual OT Hours

*What's the total overtime impact?*

450 nurses × 15 min × 260 shifts ÷ 60 = 29,250 hrs

*260 shifts = 5 shifts/week × 52 weeks average per nurse.*

## Step 3: OT Cost Eliminated

*What's the savings?*

29,250 hours × \$60 OT rate = \$203K

*OT rate = 1.5× base (\$40-50/hr ' \$60-75/hr).*

## ANNUAL VALUE

**\$203K**

## WHAT THIS MEANS

29,250 overtime hours eliminated annually—that's \$203K in direct payroll savings. This shows up immediately in your labor reports and is one of the most measurable impacts of ambient documentation.



## LABOR COST BENEFIT

### Nurse Retention

**\$95K annually**

#### WHY THIS MATTERS

Nursing turnover runs 18-25% annually—significantly higher than most hospital roles. The cost per departure is \$40-60K when you factor in recruiting, hiring, onboarding, training, and the productivity ramp for new nurses learning unit workflows.

But the real cost is harder to measure: institutional knowledge lost, team dynamics disrupted, patient relationships broken, and the burden on remaining staff who pick up extra shifts.

Why do nurses leave? The research is consistent: workload and patient ratios, schedules and work-life balance, compensation and career growth, and administrative burden and documentation.

Documentation burden shows up in every nursing satisfaction survey. It's the thing nurses didn't sign up for. They became nurses to care for patients, not to chart about caring for patients.

Here's where we're careful: nursing burnout is multifactorial. Documentation is ONE driver, not THE driver. Unlike physicians—where 50% of burnout is documentation-related—nurses face additional pressures (ratios, acuity, physical demands) that Abridge doesn't address.

We use 15% attribution for nursing (vs 30% for physicians) because we want to be honest about what documentation improvement can and can't do. It helps. It's not a silver bullet.

#### HOW WE CALCULATE THIS

##### Step 1: Annual Turnover

How many nurses leave annually?

$$450 \text{ nurses} \times 20\% \text{ turnover} = 90.0 \text{ departures}$$

Nursing turnover averages 18-25%.

##### Step 2: Burnout-Related

How many are burnout-driven?

$$90.0 \text{ departures} \times 40\% \text{ burnout} = 36.0 \text{ burnout}$$

Documentation is a top burnout driver in nursing.

##### Step 3: Abridge Impact

How many can be retained?

$$36.0 \text{ at-risk} \times 25\% \text{ impact} = 9.0 \text{ retained}$$

Conservative estimate—full impact takes 12+ months.

##### Step 4: Cost Savings

What's the value?

$$9.0 \text{ retained} \times \$50K \text{ cost} = \$95K$$

ANNUAL VALUE

\$95K

WHAT THIS MEANS

Retaining 9.0 additional nurses per year saves \$95K in replacement costs. This is a 12+ month impact—but satisfaction and engagement improvements often appear within months.

## QUALITY &amp; SAFETY BENEFIT

## HAPI Prevention

**\$250K (Potential)****WHY THIS MATTERS**

Hospital-acquired pressure injuries (HAPIs) are a quality measure, a patient safety issue, and a financial drain. CMS doesn't reimburse for them. The hospital absorbs the full cost—\$10,000 to \$50,000+ depending on severity.

HAPIs happen when: skin assessments are missed or delayed, turning schedules aren't followed, risk factors aren't communicated across shifts, and early warning signs aren't documented and acted on.

Real-time documentation can help with all of these. When assessments are charted as they happen (not hours later at end of shift), the information is available sooner. When risk factors are captured in conversation, they're less likely to be forgotten.

BUT HERE'S WHERE WE'RE CAREFUL: HAPIs are prevented through clinical care—turning, positioning, nutrition, skin care, mobility. Documentation supports this but doesn't replace it. A perfectly documented patient can still develop a pressure injury if the interventions don't happen.

The causal link between documentation and HAPI prevention is indirect: Better documentation ' better visibility ' earlier intervention ' fewer HAPIs. We believe this link is real. We've seen it in nursing units that improved documentation practices. But we can't claim a direct 1:1 relationship.

That's why we show this as "POTENTIAL VALUE"—not to diminish it, but to be intellectually honest about the causal chain.

**WHY THIS IS POTENTIAL VALUE**

This benefit has an indirect causal chain: ambient documentation ' less time charting ' more time at bedside ' better assessments ' fewer adverse events. The mechanism is well-supported in literature, but direct attribution is complex. We show this separately from hard ROI so you can evaluate it with appropriate context.

**HOW WE CALCULATE THIS****Step 1: Baseline HAPI Rate***How many HAPIs occur annually?*

$$300 \text{ beds} \times 2.0\% \text{ rate} = 72.0 \text{ HAPIs}$$

*National average HAPI rate is 2-3% of admissions.***Step 2: Prevention Potential***How many could be prevented?*

$$72.0 \text{ HAPIs} \times 10\% \text{ prevention} = 7.2 \text{ avoided}$$

*Conservative 10% through better assessment time.***Step 3: Cost Avoidance***What's the potential value?*

**7.2 avoided** × **\$20K cost** = **\$250K**

HAPI cost: \$10K-\$50K average. We use \$20K.

POTENTIAL ANNUAL VALUE

\$250K

WHAT THIS MEANS

If 7.2 HAPIs are prevented through better bedside time and assessment documentation, that's \$250K in avoided costs. This is potential value—the causal link is indirect but well-supported in literature.

## THE FULL PICTURE

*What else gets better—and why we don't put a number on it*

Your ROI model captures outcomes we can measure with confidence: overtime, retention, agency spend, and (with caveats) patient safety events. But ambient documentation also improves areas that are harder to quantify—and often matter just as much to your organization.

We're showing these separately because we want to be honest about what we can and can't measure. These benefits are real. They show up in surveys, in audits, in patient feedback, in the daily experience of nurses. But attributing a dollar value would require assumptions we're not comfortable making.

### Patient Experience (HCAHPS)

Patients notice when nurses are fully present versus distracted by documentation. The research is clear: nurse communication scores correlate with bedside presence. When nurses chart at the bedside on a laptop, patients perceive divided attention. When nurses are fully present—making eye contact, listening actively, explaining clearly—scores improve.

Ambient documentation removes the laptop from the interaction. The nurse can focus on the patient. The charting happens automatically.

*Impacts: "Nurse listened carefully" scores, "Nurse explained things" ratings, overall nurse communication domain, VBP reimbursement tied to HCAHPS*

*Why we don't quantify: HCAHPS is influenced by dozens of factors—staffing, acuity, room cleanliness, food quality, pain management. Isolating the documentation effect is nearly impossible.*

### Survey & Compliance Readiness

Complete documentation is your first line of defense in any survey. Joint Commission, CMS, state surveys—they all start with the chart. When documentation is complete, timely, and accurate, surveyors find what they're looking for. When it's not, you get findings, corrective action plans, and follow-up visits.

Real-time documentation means charts are always current. There's no end-of-shift catch-up. No "I'll finish that later." The documentation exists because the conversation happened.

*Impacts: Survey deficiency rates, time spent preparing for surveys, remediation costs after findings, staff anxiety around survey readiness*

*Why we don't quantify: Survey outcomes are binary and infrequent. Attributing a dollar value would require assumptions about deficiency probability that vary too much by organization.*

### Care Coordination

Handoffs are only as good as the documentation behind them. Shift-to-shift handoffs, department-to-department transfers, discharge planning—they all depend on complete, accurate, timely documentation. When documentation lags, information gets lost. When it's real-time, everyone works from the same current picture.

*Impacts: Shift-to-shift handoff quality, interdepartmental communication, miscommunication-related safety events, time spent "hunting" for information*

*Why we don't quantify: Care coordination benefits are diffuse and hard to isolate. Better handoffs lead to fewer errors, faster care, better outcomes—but the causal chain is long and confounded.*

### Why This Matters

We could have assigned dollar values to these outcomes. Other vendors do. But we'd rather give you a conservative ROI you can defend than an inflated one that falls apart under scrutiny.

**These outcomes are real—they show up in surveys, in patient feedback, in the daily experience of nurses. And they matter to your board, your CNO, and your patients.**

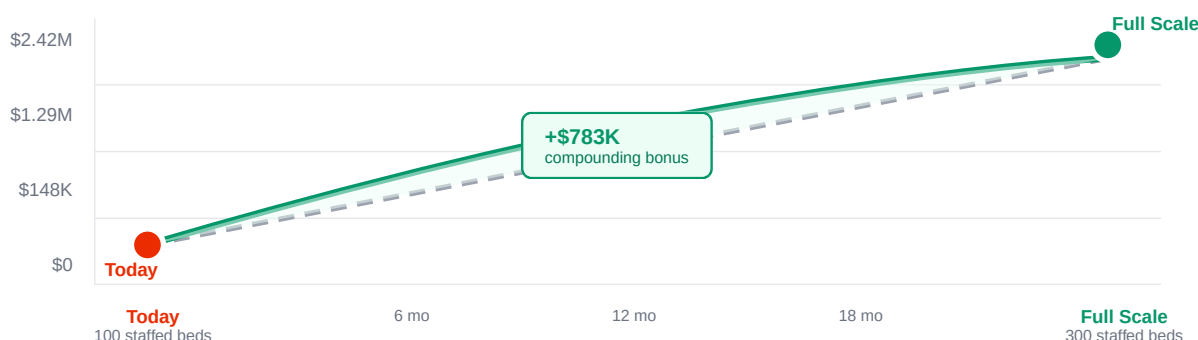


## YOUR VALUE JOURNEY

From pilot to full scale

**Implementation follows a proven path.** Starting with a pilot group of 100 staffed beds, you'll prove value quickly before expanding. This isn't just about adding more units—it's about building compounding returns as adoption increases.

At full scale with 300 staffed beds and 80% utilization, annual value reaches \$2.42M—a 16.4x increase from pilot. The journey matters as much as the destination.



### Pilot Phase

- 100 staffed beds
- 189,800 documentation events
- 65% utilization target
- Annual value: **\$148K**

### Full Scale

- 300 staffed beds
- 80% utilization target
- 24 months timeline
- Annual value: **\$2.42M**

## THE COMPOUNDING EFFECT

# +\$783K

Additional annual value from network effects at full scale

**Value doesn't grow linearly—it compounds.** As more units adopt Abridge, documentation quality improves floor by floor. Overtime patterns shift. Agency reliance drops. The value per bed increases even as you add beds.

This is why pilot success often understates full-scale potential. At 80% utilization with 300 staffed beds, network effects add \$783K to your annual value—bringing total potential to \$3.21M.

**CONNECTED VALUE***Nursing + Inpatient compounds your results*

This assessment models nursing-specific value. Organizations implementing Abridge across nursing and inpatient settings see compounding benefits: nurses charting faster means hospitalists get better context, CDI has cleaner records to work with, and the entire documentation ecosystem improves.

**METHODOLOGY & ASSUMPTIONS***How we built your model***YOUR INPUTS**

300 staffed beds  
450 nurse FTEs  
80% expected utilization  
5 min saved per documentation event  
\$225/staffed bed/month

**LABOR COST BENCHMARKS**

| METRIC              | RANGE           | DEFAULT          |
|---------------------|-----------------|------------------|
| RN Base Hourly Rate | \$40-\$50       | <b>\$45</b>      |
| OT Rate (1.5x)      | \$60-\$75       | <b>\$60</b>      |
| Agency Premium      | \$65K-\$85K/FTE | <b>\$75K/FTE</b> |
| Replacement Cost    | \$40K-\$60K     | <b>\$50K</b>     |
| Turnover Rate       | 18-25%          | <b>20%</b>       |

**QUALITY & SAFETY BENCHMARKS**

| METRIC    | RANGE       | DEFAULT        |
|-----------|-------------|----------------|
| HAPI Cost | \$10K-\$50K | <b>\$20K</b>   |
| Fall Cost | \$3K-\$30K  | <b>\$6,500</b> |
| HAPI Rate | 2-3%        | <b>2%</b>      |
| Fall Rate | 3-5%        | <b>3%</b>      |

**Conservative by Design**

Every assumption here can be adjusted. We start conservative because credibility matters more than inflated projections. Your actual results may differ—often in positive ways we didn't model.

*"Nursing documentation carries a different kind of weight—it's not just about coding or billing. It's about **patient safety**, **regulatory compliance**, and the daily experience of nurses who spend 25-35% of their shift charting instead of caring. The ROI is measurable. The impact on culture is harder to quantify—but often matters more."*

**This assessment represents a starting point.** The calculations here are based on industry benchmarks and conservative assumptions. Your organization's actual experience will depend on current documentation workflows, nurse engagement, and implementation approach.

What this model doesn't capture: the relief on nurses' faces when they realize they can go home on time. The quality of patient interactions when nurses aren't typing. The cultural shift when documentation stops being a burden.

Those outcomes matter. They're just harder to put in a spreadsheet.

YOUR SUMMARY

|                            |        |
|----------------------------|--------|
| Total Annual Value (Labor) | \$297K |
| Potential Value (Quality)  | \$250K |
| Annual Investment          | \$270K |
| Net Value Created          | \$27K  |